Leuprorelin 22.5 mg: hormone injection every 12 weeks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

One injection every 12 weeks

Active medicines: Leuprorelin 22.5 mg

Emetogenic risk: Minimal

No routine prophylactic antiemetic order. Agree symptom treatment if needed.

Additional phase-specific support

- A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.
- Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.
- Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.
- Bone health, calcium and vitamin D according to individual needs and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-14 · SACTCheck v0.84.52 · Regimen-level drafts; prescriber and oncology-pharmacy verification required